# Tucatinib + capecitabine + trastuzumab

Tucatinib twice daily every day; capecitabine twice daily on Days 1–14 then off on Days 15–21; trastuzumab every 21 days by IV infusion or a locally approved under-the-skin injection. Continue while beneficial and tolerated, with clinician-approved changes.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

## A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

### 1 Diarrhoea / dehydration

**Tucatinib + capecitabine**

Report diarrhoea early. Follow the separate tablet-hold instructions; reduced urine or dizziness needs prompt advice.

### 3 Liver injury

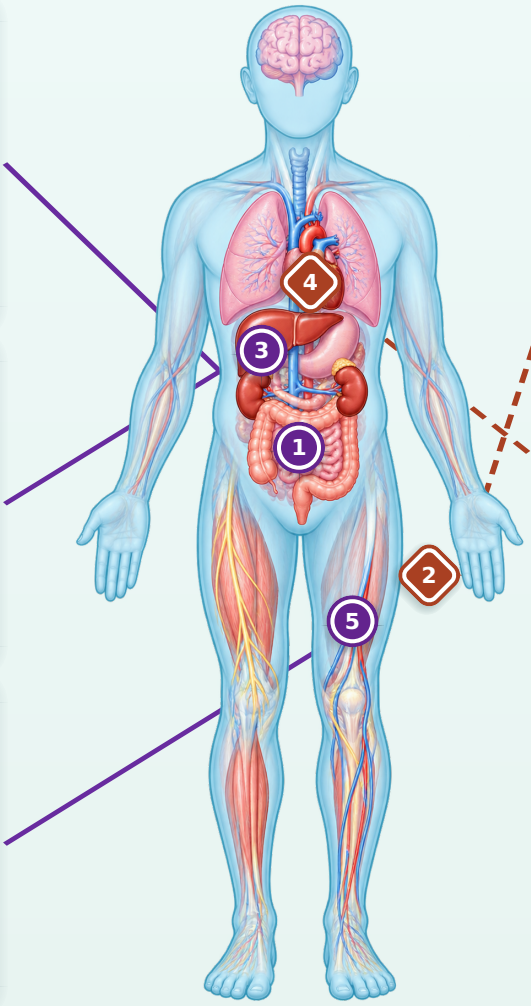
**Tucatinib**

Keep three-weekly liver blood tests. Report yellow skin / eyes or dark urine.

### 5 Blood counts / infection

**Capecitabine**

Fever, shivers or sudden illness: call promptly. Blood tests guide the next cycle.




### 2 Hands / feet

**Capecitabine**

Painful redness or peeling affecting activity: stop capecitabine and contact oncology.

### 4 Heart / chest symptoms

**Trastuzumab / capecitabine**

Keep echoes. New chest pain: stop capecitabine and seek urgent assessment.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

# Your treatment, at home.

## Important medicine-specific effects

### Tucatinib

- Diarrhoea and dehydration
- Liver test changes or liver injury
- Nausea and fatigue
- Clinically important medicine interactions

### Capecitabine

- Diarrhoea / dehydration
- Hand-foot syndrome
- Mouth ulcers / reduced intake
- Low blood counts and infection
- Cardiotoxicity including chest pain / ischaemia
- Severe early toxicity including DPD-related toxicity
- Renal impairment and medicine interactions

### Trastuzumab

- Heart pumping-function reduction / heart failure
- Infusion or allergic reaction
- Diarrhoea and fatigue
- Rare serious lung effects

**Surgery, radiation and monitoring:** open the whole-treatment map or scan the QR.

## Make the plan practical

### Support for this phase

Obtain a clear diarrhoea action plan and separate instructions for each tablet. Anti-sickness medicines and interactions require individual review.

### Bloods and cardiac checks

FBC, renal, liver; DPD testing before capecitabine → Extra tests if symptoms / toxicity → Pre-cycle bloods and symptom review. Symptoms need assessment between visits.

### Visits and practical support

Ask about travel, work, fertility, menopause, exercise and help at home. The team arranges and reviews clinical checks; the patient should not have to calculate the monitoring timetable.

### Follow-up and bone health

If an aromatase inhibitor or ovarian suppression is prescribed, assess fracture risk and baseline DEXA as indicated. Repeat timing depends on the result, risk and bone treatment; it is not an automatic scan for everyone.

**Emergency: 112 / 999.** Severe breathing difficulty, collapse, seizure or severe chest pain needs emergency help.

**Contact oncology immediately** for temperature ≥37.5°C or ≤36°C, shivering or suddenly feeling unwell, even with a normal temperature. Follow your own hospital's instructions. If you cannot reach the team, seek urgent medical assessment.

YOUR ONCOLOGY TEAM · DAYTIME

**Insert numbers here**

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

**Insert numbers here**

Ask for medical oncology

EVENINGS / OVERNIGHT

**Your out-of-hours pathway / nearest ED**

Other hospitals: use the numbers your team provides.

**Complete reference, visual timeline and passport**

https://sactcheck.com/patient/00936/

Sources and review status are available online. Follow your own prescription and hospital instructions.